



Anthony Michael Dimarcello "Anthony"

Patient Health Summary, generated on Aug. 28, 2026

Patient Demographics - Male; born May 04, 1983

Patient Address	Patient Name	Communication
412 Olive Ave (Home) Huntington Beach, CA 92648-5142	Anthony Michael Dimarcello	949-350-2312 (Mobile) amd949609@gmail.com
Former (Aug. 17, 2026 - Aug. 17, 2026): 412 Olive Ave (Home) Huntington Beach, CA 92648	Former / Aliases: Anthony Dimarcello	
Former (Aug. 17, 2026 - Aug. 16, 2026): 412 Olive Ave (Home) Huntington Beach, B.C. 92648		
Former (Mar. 17, 2026 - Aug. 16, 2026): 412 Olive Ave (Home) Huntington Beach, CA 92648		
Former (Mar. 17, 2026 - Mar. 16, 2026): (Home)		
Language	Race / Ethnicity	Marital Status
English (Preferred)	White / Not Hispanic or Latino	Never Married

Note from UCI Health

This document contains information that was shared with Anthony Michael Dimarcello. It may not contain the entire record from UCI Health.

Allergies

No known active allergies

Medications

No known medications

Active Problems

Not on file

Social History

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Never				
Smokeless Tobacco: Current				
Tobacco Cessation: Ready to Quit: Not Asked; Counseling Given: Not Answered				
Alcohol Use	Standard Drinks/Week			
Not Currently	0 (1 standard drink = 0.6 oz pure alcohol)			
Sex and Gender Information		Value	Date Recorded	
Sex Assigned at Birth		Not on file		
Legal Sex		Male	12/14/2025 10:59 AM PST	
Gender Identity		Male	08/20/2026 12:01 PM PDT	
Sexual Orientation		Not on file		

Last Filed Vital Signs

Vital Sign	Reading	Time Taken	Comments
Blood Pressure	118/75	12/14/2025 4:45 PM PST	
Pulse	75	12/14/2025 4:45 PM PST	
Temperature	37 °C (98.6 °F)	12/14/2025 4:45 PM PST	
Respiratory Rate	16	12/14/2025 4:45 PM PST	
Oxygen Saturation	99%	12/14/2025 4:45 PM PST	
Inhaled Oxygen Concentration	-	-	
Weight	81.6 kg (180 lb)	12/14/2025 11:16 AM PST	
Height	180.3 cm (5' 11")	12/14/2025 11:16 AM PST	
Body Mass Index	25.1	12/14/2025 11:16 AM PST	

Procedures

TRANSTHORACIC ECHO (TTE) (Performed 12/14/2025)

LACERATION REPAIR (Performed 12/14/2025)

Results

TRANSTHORACIC ECHO (TTE) - Final result (12/14/2025 4:00 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
LV Stroke Volume Index	37.8	ml/m2			CPACS	
AO Peak Velocity	1.34	m/s			CPACS	
LV Diastolic Diameter	5.36	cm			CPACS	
Mitral Valve Area	4.0	mmHg			CPACS	
Aortic Valve Area	1.98	cm2			CPACS	
Mitral Valve Area	3.50	cm2			CPACS	
LV Ejection Fraction	62.1	%			CPACS	
LA Volume Index	29.2	ml/m2			CPACS	

Anatomical Region	Laterality	Modality
Cardiac/Heart		Echography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/14/2025 3:29 PM PST	

Narrative

12/14/2025 4:14 PM PST

UCI Health
Cardiac Diagnostic Center Echocardiography Report

Name:ANTHONY MICHAEL

MRN #:5405781

ACC #:251317823

Location:IP ED STAT

Billing Location Orange 80102067.

Sex: M

DOB: 5/4/1983

Ht: 180.30 cm

Wt: 81.65 kg

Date/Time: 12/14/2025 3:29:50 PM

Age: 42 years

BP: 110 / 78 mmHg

BSA: 2.02 m²

Heart Rate: 72 bpm

Referring Physician: 112276 SHANNON L TOOHEY , cc:

Indications: Syncope

Procedure Codes: Complete echo without contrast with spectral strain and 3D - 4839330602;- 4809335600;- 3507637601;- 93306;- 93356;- 76376

Technical Quality: Image quality for this study is adequate.

Appropriate Use Criteria: Symptoms or conditions potentially related to suspected cardiology etiology; AUC score = 9.

2D AND M-MODE MEASUREMENTS (normal ranges within parentheses):

Left ventricle: Normal Aorta/Left Atrium: Normal

IVSd (2D): 1.11 cm (0.7-1.1) Aortic Sinus: 3.53 cm

LVPwd (2D): 0.97 cm (0.7-1.1) Left Atrium (2D): 2.60 cm (1.9-4.0)

LVIDd (2D): 5.36 cm (3.4-5.7) LA Vol Index A/L:

Narrative

LVIDs (2D): 3.95 cm LA Vol Index A4C 19.6 ml/mA²
LVEDV Index 60.79 LA Vol Index A2C 34.9 ml/mA²
LA Vol Index BP 29.2 ml/mA²
Right Ventricle: RA Vol Index A/L:
Rvd A4C base(2D): 3.16 cm RA Vol Index A4C 24.0 ml/mA²

LV SYSTOLIC FUNCTION BY 2D PLANIMETRY (MOD):
EF-Biplane: 62.1 %
LV stroke volume index-Biplane: 37.8 ml/mA²
LV GLS: -19.3 %

LV DIASTOLIC FUNCTION:
MV Peak E: 0.57 m/s e', MV Ann: 0.12 m/s MV A Dur 116 msec
MV Peak A: 0.46 m/s E/e' Ratio: 4.56
E/A Ratio: 1.25 Decel Time: 217 msec

SPECTRAL DOPPLER ANALYSIS (where applicable)
MV P1/2 Time: 62.93 msec
MV Area, PHT: 3.50 cmA²
MR VTI:

Right Ventricle (RV) Function by 2D: RV TAPSE: 2.1 cm

Aortic Valve: AoV Max Velocity: 1.34 m/s AoV Peak PG: 7.2 mmHg AoV Mean PG: 4.0 mmHg

LVOT Vmax: 0.91 m/s LVOT VTI: 0.150 m LVOT Diameter: 1.90 cm

AoV Area, Vmax: 1.93 cmA² AoV Area, VTI: 1.98 cmA² AoV Area, Vmn: 2.05 cmA²
AV AT: AVAI: 0.98 cmA²/mA² DVI:

Tricuspid Valve and PA/RV Systolic Pressure: TR Max Velocity: RA Pressure: 3 mmHg RVSP/PASP:

Pulmonic Valve:
PV Max Velocity: 1.02 m/s PV Max PG: 4.2 mmHg PV Mean PG:

PHYSICIAN INTERPRETATION:

Left Ventricle: The left ventricular internal size is normal. No evidence of left ventricular hypertrophy. There is global Low normal left ventricular systolic function. The left ventricular ejection fraction is 52% by 3D. Left ventricular ejection fraction is normal diastolic left ventricular function is noted. Normal LV filling pressures. Global longitudinal strain is normal.

LV Wall Scoring:
All segments are normal.

Right Ventricle: Normal right ventricular size and systolic function.

Left Atrium: The left atrium is normal in size and structure.

Right Atrium: The right atrium is normal in size and structure.

Mitral Valve: Structurally normal mitral valve, with normal leaflet excursion. No evidence of mitral valve regurgitation is seen.

Tricuspid Valve: The tricuspid valve is normal in structure. No tricuspid regurgitation was seen.

Aortic Valve: The aortic valve is tricuspid. The aortic valve is without evidence of sclerosis or stenosis. No evidence of aortic valve regurgitation is seen.

Pulmonic Valve: The pulmonic valve was not well visualized. No indication of pulmonic valve regurgitation. No evidence of pulmonic stenosis.

Aorta: The aortic root is normal in size and structure.

Pericardium: There is no evidence of pericardial effusion.

Pulmonary Artery: Inadequate Doppler signal to assess pulmonary artery systolic pressure.

Venous: A normal flow pattern is recorded from the right upper pulmonary vein. The inferior vena cava is normal in size and exhibits greater than 50% respiratory size variation.

Summary:

1. Low normal left ventricular systolic function. The left ventricular ejection fraction is 52% by 3D
2. Normal diastolic LV function.
3. Normal LV filling pressures.
4. Normal right ventricular size and systolic function.

In comparison to the previous echocardiogram(s): There are no prior studies on this patient for comparison purposes.

122986 Behram Mody MD Sonographer: BT
UCI Division of Cardiology

Electronically signed by 122986 Behram Mody MD on 12/14/2025 at 4:14:30 PM
ICD CODES: Syncope

Final

Dr. Behram Mody, MD - 12/14/2025

Formatting of this note might be different from the original.

UCI Health

Cardiac Diagnostic Center Echocardiography Report

Name: ANTHONY MICHAEL Sex: M Date/Time: 12/14/2025 3:29:50

DIMARCELLO PM

MRN #: 5405781 DOB: 5/4/1983 Age: 42 years

ACC #: 251317823 Ht: 180.30 BP: 110 / 78 mmHg

cm

Location: IP ED STAT Wt: 81.65 kg BSA: 2.02 m² Heart Rate: 72

bpm

Billing Location Orange 80102067.

Referring Physician: 112276 SHANNON L TOOHEY , cc:

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Right Atrium: The right atrium is normal in size and structure.
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Tricuspid Valve: The tricuspid valve is normal in structure. No tricuspid regurgitation was seen.
Aortic Valve: The aortic valve is tricuspid. The aortic valve is without evidence of sclerosis or stenosis. No evidence of aortic valve regurgitation is seen.
Pulmonic Valve: The pulmonic valve was not well visualized. No indication of pulmonic valve regurgitation. No evidence of pulmonic stenosis.
Aorta: The aortic root is normal in size and structure.
Pericardium: There is no evidence of pericardial effusion.
Pulmonary Artery: Inadequate Doppler signal to assess pulmonary artery systolic pressure.
Venous: A normal flow pattern is recorded from the right upper pulmonary vein. The inferior vena cava is normal in size and exhibits greater than 50% respiratory size variation.

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 - 2. Normal diastolic LV function.
 - 3. Normal LV filling pressures.
 - 4. Normal right ventricular size and systolic function.

In comparison to the previous echocardiogram(s): There are no prior studies on this patient for comparison purposes.

122986 Behram Mody MD Sonographer: BT
UCI Division of Cardiology

Electronically signed by 122986 Behram Mody MD on 12/14/2025 at 4:14:30 PM
ICD CODES: Syncope

Final

Authorizing Provider	Result Type	Result Status
Dr. Shannon Toohey MD	CV ECHO PROCEDURES	Final Result

CT C-Spine W/O Contrast (CT C-SPINE W/O CONTRAST) - Final result (12/14/2025 2:54 PM PST)

Anatomical Region	Laterality	Modality
Spine, C-spine, Musculoskeletal		Computed Tomography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/14/2025 2:23 PM PST	

Impressions
12/14/2025 2:36 PM PST

Impressions

No acute intracranial hemorrhage, midline shift, herniation, or hydrocephalus.

Scalp injury without underlying acute fracture.

No acute cervical spine fracture.

I have personally reviewed the images upon which this report is based and agree with the findings and conclusions expressed above.

Narrative

12/14/2025 2:36 PM PST

CT HEAD W/O CONTRAST, CT C-SPINE W/O CONTRAST

INDICATION: SYNCOPE, head trauma

EXAM DATE: 12/14/2025 2:19 PM

COMPARISON: None

TECHNIQUE:

CT head without intravenous contrast.

CT cervical spine without intravenous contrast.

RADIATION DOSE:

This patient received a total of 2 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 203; Anatomy: C Spine; Phantom: 32 cm; CTDIvol: 11; DLP: 298

Exposure: 2; Series: 303; Anatomy: C Spine; Phantom: 16 cm; CTDIvol: 46; DLP: 1021

The following accession numbers are related to this dose report {251317997}:
251318064

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient. CT scan was completed using equipment that meets the NEMA XR-29 safety standards.

(accession 251317997),

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(accession 251318064)

FINDINGS:

CT head:

Hemorrhage: None.

Brain parenchyma: Unremarkable.

Mass effect: No midline shift. No herniation. No hydrocephalus.

Osseous/extracranial soft tissues: No acute calvarial fracture. Left posterior scalp injury with skin staples.

CT cervical spine:

Alignment: Straightening of the usual cervical lordosis.

vertebral bodies and posterior elements: No acute fracture. Intact heights. No suspicious lesion.

Discs and joints: Mild degenerative changes.

Craniocervical junction: Intact.

Soft tissues: No prevertebral edema. Intact surrounding soft tissues.

Additional findings: None.

Formatting of this note might be different from the original.

CT HEAD W/O CONTRAST, CT C-SPINE W/O CONTRAST

INDICATION: SYNCOPE, head trauma

EXAM DATE: 12/14/2025 2:19 PM

COMPARISON: None

TECHNIQUE:

CT head without intravenous contrast.

CT cervical spine without intravenous contrast.

RADIATION DOSE:

This patient received a total of 2 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

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(accession 251318064)

FINDINGS:

CT head:

Hemorrhage: None.

Brain parenchyma: Unremarkable.

Mass effect: No midline shift. No herniation. No hydrocephalus.

Osseous/extracranial soft tissues: No acute calvarial fracture. Left posterior scalp injury with skin staples.

CT cervical spine:

Alignment: Straightening of the usual cervical lordosis.

Vertebral bodies and posterior elements: No acute fracture. Intact heights. No suspicious lesion.

Discs and joints: Mild degenerative changes.

Procedure Note

Craniocervical junction: Intact.

Soft tissues: No prevertebral edema. Intact surrounding soft tissues.

Additional findings: None.

IMPRESSION:

No acute intracranial hemorrhage, midline shift, herniation, or hydrocephalus.

Scalp injury without underlying acute fracture.

No acute cervical spine fracture.

I have personally reviewed the images upon which this report is based and agree with the findings and conclusions expressed above.

Authorizing Provider	Result Type	Result Status
Dr. Shannon Toohey MD	IMG CT PROCEDURES	Final Result

CT Head W/O Contrast (CT HEAD W/O CONTRAST) - Final result (12/14/2025 2:54 PM PST)

Anatomical Region	Laterality	Modality
Head and Neck		Computed Tomography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/14/2025 2:23 PM PST	

Impressions

12/14/2025 2:36 PM PST

No acute intracranial hemorrhage, midline shift, herniation, or hydrocephalus.

Scalp injury without underlying acute fracture.

No acute cervical spine fracture.

I have personally reviewed the images upon which this report is based and agree with the findings and conclusions expressed above.

Narrative

12/14/2025 2:36 PM PST

CT HEAD W/O CONTRAST, CT C-SPINE W/O CONTRAST

INDICATION: SYNCOPE, head trauma

EXAM DATE: 12/14/2025 2:19 PM

COMPARISON: None

TECHNIQUE:

CT head without intravenous contrast.

CT cervical spine without intravenous contrast.

RADIATION DOSE:

This patient received a total of 2 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 203; Anatomy: C Spine; Phantom: 32 cm; CTDIvol: 11; DLP: 298

Exposure: 2; Series: 303; Anatomy: C Spine; Phantom: 16 cm; CTDIvol: 46; DLP: 1021

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(accession 251318064)

FINDINGS:

CT head:

Hemorrhage: None.

Brain parenchyma: Unremarkable.

Mass effect: No midline shift. No herniation. No hydrocephalus.

Osseous/extracranial soft tissues: No acute calvarial fracture. Left posterior scalp injury with skin staples.

CT cervical spine:

Alignment: Straightening of the usual cervical lordosis.

vertebral bodies and posterior elements: No acute fracture. Intact heights. No suspicious lesion.

Discs and joints: Mild degenerative changes.

Cranio cervical junction: Intact.

Soft tissues: No prevertebral edema. Intact surrounding soft tissues.

Additional findings: None.

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CT HEAD W/O CONTRAST, CT C-SPINE W/O CONTRAST

INDICATION: SYNCOPE, head trauma

EXAM DATE: 12/14/2025 2:19 PM

COMPARISON: None

TECHNIQUE:

CT head without intravenous contrast.

CT cervical spine without intravenous contrast.

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Authorizing Provider	Result Type	Result Status
Dr. Shannon Toohey MD	IMG CT PROCEDURES	Final Result

TROPONIN I, HIGH SENSITIVITY (ED) - UCI - Final result (12/14/2025 2:16 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I, High Sensitivity	<3	0 - 20 ng/L		12/14/2025 3:04 PM PST	UCI MEDICAL CENTER	
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time		
Blood	Peripheral blood / Unknown	Venipuncture / Unknown	12/14/2025 2:16 PM PST	12/14/2025 2:35 PM PST		

Narrative						
UCI MEDICAL CENTER - 12/14/2025 3:04 PM PST						
These values were determined with a high sensitivity troponin I assay. The 99th percentile upper reference limits established by the manufacturer are:						
Females: 15 ng/L						
Males: 20 ng/L						
Overall: 18 ng/L						

Authorizing Provider	Result Type	Result Status	
Dr. Shannon Toohey MD	LAB BLOOD ORDERABLES	Final Result	
Performing Organization	Address	City/State/ZIP Code	Phone Number
UCI MEDICAL CENTER	101 The City Drive	Orange, CA 92868	

ECG 12-LEAD - Edited Result - FINAL (12/14/2025 1:44 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Heart Rate	62	bpm			FOUNDATION RADIOLOGY SYSTEM	
RR Interval	968	ms			FOUNDATION RADIOLOGY SYSTEM	
Atrial Rate	63	ms			FOUNDATION RADIOLOGY SYSTEM	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
PR Interval	157	ms			FOUNDATION RADIOLOGY SYSTEM	
P Front Axis	63	deg			FOUNDATION RADIOLOGY SYSTEM	
QRSD Interval	93	ms			FOUNDATION RADIOLOGY SYSTEM	
QT Interval	456	ms			FOUNDATION RADIOLOGY SYSTEM	
QTcF	461	ms			FOUNDATION RADIOLOGY SYSTEM	
QTcFm	460	ms			FOUNDATION RADIOLOGY SYSTEM	
QRS Axis	60	deg			FOUNDATION RADIOLOGY SYSTEM	
T Wave Axis	81	deg			FOUNDATION RADIOLOGY SYSTEM	
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume		Collection Time	Received Time	
				12/14/2025 1:43 PM PST		

Narrative

FOUNDATION RADIOLOGY SYSTEM - 12/15/2025 10:16 PM PST
This result has an attachment that is not available.
- BORDERLINE ECG -
Sinus rhythm
Borderline T wave abnormalities
No previous ECG available for comparison

Procedure Note

Dr. Satinder Swaroop, MD - 01/20/2026
Formatting of this note might be different from the original.
- BORDERLINE ECG -
Sinus rhythm
Borderline T wave abnormalities
No previous ECG available for comparison

Authorizing Provider	Result Type	Result Status	
Dr. Shannon Toohey MD	ECG ORDERABLES	Edited Result - Final	
Performing Organization	Address	City/State/ZIP Code	Phone Number
FOUNDATION RADIOLOGY SYSTEM	123 Anywhere Street	Madison, WI 53593, US	

TROPONIN I, HIGH SENSITIVITY (ED) - UCI - Final result (12/14/2025 12:44 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I, High Sensitivity	<3	0 - 20 ng/L		12/14/2025 1:40 PM PST	UCI MEDICAL CENTER	
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume		Collection Time	Received Time	
Blood	Peripheral blood / Unknown	Venipuncture / Unknown		12/14/2025 12:44 PM PST	12/14/2025 1:10 PM PST	

Narrative

UCI MEDICAL CENTER - 12/14/2025 1:40 PM PST

These values were determined with a high sensitivity troponin I assay. The 99th percentile upper reference limits established by the manufacturer are:

Females: 15 ng/L
Males: 20 ng/L
Overall: 18 ng/L

Authorizing Provider	Result Type	Result Status	
Dr. Shannon Toohey MD	LAB BLOOD ORDERABLES	Final Result	
Performing Organization	Address	City/State/ZIP Code	Phone Number
UCI MEDICAL CENTER	101 The City Drive	Orange, CA 92868	

LACERATION REPAIR (Laceration Repair) - Final result (12/14/2025 12:20 PM PST)

Narrative

Dr. Shannon Toohey, MD - 12/14/2025 12:20 PM PST

Toohey, Shannon L, MD 12/15/2025 1:08 PM
Laceration Repair

Date/Time: 12/14/2025 12:20 PM

Performed by: Hua, Christine Tran, MD
Authorized by: Hua, Christine Tran, MD
Consent Obtained: Verbal
Consent given by: Patient
Risks and benefits: Risks, benefits and alternatives were discussed

Indication: Laceration

Consent: Prior to the procedure, the risks, benefits and alternatives were explained to the patient. The patient gave verbal consent to the procedure.

Timeout: Prior to the procedure, a time-out was called and correct patient and procedure were confirmed.

Site: left occipital scalp

Procedure: Prior to the repair, the patient's wound was cleansed. A total of 50 ml of normal saline was used to thoroughly irrigate the wound under pressure. The wound was inspected and no obvious foreign objects were found. The patient's wound was repaired with 6 staples. Total wound length was 3 cm. The patient tolerated the procedure well and there were no immediate complications.

Supervised Procedure: I am a resident/fellow and an attending was present to supervise the procedure
patient tolerated the procedure well with no immediate complications

Authorizing Provider	Result Type	Result Status
Christine Hua MD	IN CLINIC/BEDSIDE ORDERABLES	Final Result

X-Ray Chest Single View (X-RAY CHEST SINGLE VIEW) - Final result (12/14/2025 12:13 PM PST)

Anatomical Region	Laterality	Modality		
Chest		Digital Radiography		
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/14/2025 12:45 PM PST	

Impressions

12/14/2025 3:21 PM PST

Impressions

FINDINGS/

Frontal view chest. Clear lungs. No pleural effusion or pneumothorax. Normal cardiomediastinal silhouette.

No acute osseous abnormality.

Narrative

12/14/2025 3:21 PM PST

EXAM: X-RAY CHEST SINGLE VIEW

DATE: 12/14/2025 12:09 PM

HISTORY: SYNCOPE.

COMPARISON: None available at time of dictation.

Procedure Note

Dr. Anderanik Tomasian, MD - 12/14/2025

Formatting of this note might be different from the original.

EXAM: X-RAY CHEST SINGLE VIEW

DATE: 12/14/2025 12:09 PM

HISTORY: SYNCOPE.

COMPARISON: None available at time of dictation.

IMPRESSION:
FINDINGS/

Frontal view chest. Clear lungs. No pleural effusion or pneumothorax. Normal cardiomediastinal silhouette.

No acute osseous abnormality.

Authorizing Provider	Result Type	Result Status
Dr. Shannon Toohey MD	IMG XR PROCEDURES	Final Result

TROPONIN I, HIGH SENSITIVITY (ED) - UCI - Final result (12/14/2025 11:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I, High Sensitivity	<3	0 - 20 ng/L		12/14/2025 12:25 PM PST	UCI MEDICAL CENTER	
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time		
Blood	Peripheral blood / Unknown	Venipuncture / Unknown	12/14/2025 11:45 AM PST	12/14/2025 11:54 AM PST		

Narrative

UCI MEDICAL CENTER - 12/14/2025 12:25 PM PST

These values were determined with a high sensitivity troponin I assay. The 99th percentile upper reference limits established by the manufacturer are:

Females: 15 ng/L
Males: 20 ng/L
Overall: 18 ng/L

Authorizing Provider	Result Type	Result Status	
Dr. Shannon Toohey MD	LAB BLOOD ORDERABLES	Final Result	
Performing Organization	Address	City/State/ZIP Code	Phone Number
UCI MEDICAL CENTER	101 The City Drive	Orange, CA 92868	

(ABNORMAL) PT/INR/PTT - UCI - Final result (12/14/2025 11:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Prothrombin Time	11.9	10.2 - 12.9 sec		12/14/2025 12:22 PM PST	UCI MEDICAL CENTER	
INR	1.03	0.88 - 1.12		12/14/2025 12:22 PM PST	UCI MEDICAL CENTER	

Comment:
Therapy guidelines using the INR for patients stabilized on oral anticoagulant therapy:
 Moderate Intensity Regimen
 2.0 - 3.0 INR
 High Intensity Regimen
 2.5 - 3.5 INR
The physician must determine the best INR range depending on the reason for anticoagulant treatment and how each individual responds to treatment.
Reference: Chest 133:160S-198S, 2008

APTT	25.0 (L)	25.1 - 36.5 sec		12/14/2025 12:22 PM PST	UCI MEDICAL CENTER	
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Comment:
Therapeutic Range for DVT or PE:
- Unfractionated Heparin: 1.5-2.5x of the mean aPTT reference range, 6 hours after bolus
- Argatroban: 1.5-3x of the mean aPTT reference range
- Please refer to latest pharmacy dosing protocol for current numeric ranges.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Peripheral blood / Unknown	Venipuncture / Unknown	12/14/2025 11:45 AM PST	12/14/2025 11:54 AM PST

Narrative

Authorizing Provider	Result Type	Result Status	
Dr. Shannon Toohey MD	LAB BLOOD ORDERABLES	Final Result	
Performing Organization	Address	City/State/ZIP Code	Phone Number
UCI MEDICAL CENTER	101 The City Drive	Orange, CA 92868	

DIFFERENTIAL - UCI - Final result (12/14/2025 11:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Diff Type	Differential performed by automated analysis.			12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Neutrophils %	75.0	39.0 - 88.0 %		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Neutrophils Absolute	7.0	2.0 - 8.1 thous/ mcL		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Lymphocytes %	18.0	14.0 - 52.0 %		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Lymphocytes Absolute	1.7	0.9 - 3.3 thous/ mcL		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Monocytes %	5.6	1.0 - 11.0 %		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Monocytes Absolute	0.5	0.0 - 0.8 thous/ mcL		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Eosinophils %	0.9	0.0 - 7.0 %		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Eosinophils Absolute	0.1	0.0 - 0.5 thous/ mcL		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Basophils %	0.5	0.0 - 2.0 %		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Basophils Absolute	0.1	0.0 - 0.2 thous/ mcL		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Specimen (Source)	Anatomical Location / Laterality		Collection Method / Volume	Collection Time		Received Time
Blood	Peripheral blood / Unknown		Venipuncture / Unknown	12/14/2025 11:45 AM PST		12/14/2025 11:54 AM PST
Narrative						
Authorizing Provider	Result Type		Result Status			
Dr. Shannon Toohey MD	LAB BLOOD ORDERABLES		Final Result			
Performing Organization	Address		City/State/ZIP Code	Phone Number		
UCI MEDICAL CENTER	101 The City Drive		Orange, CA 92868			
HEMOGRAM, BLOOD - UCI - Final result (12/14/2025 11:45 AM PST)						
Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	9.3	4.0 - 10.5 thous/ mcL		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
RBC	5.09	4.38 - 5.62 mill/ mcl		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
HGB	15.3	13.5 - 16.9 g/ dL		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
HCT	45.7	39.5 - 50.0 %		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
MCV	89.7	81.5 - 97.0 fl		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
MCH	30.1	27.0 - 33.5 pg		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
MCHC	33.5	32.0 - 35.5 g/dL		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
RDW-CV	13.0	11.6 - 14.4 %		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
PLT Count	227	150 - 400 thous/mcl		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
MPV	8.2	7.2 - 11.7 fl		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Peripheral blood / Unknown	Venipuncture / Unknown	12/14/2025 11:45 AM PST	12/14/2025 11:54 AM PST

Narrative

Authorizing Provider	Result Type	Result Status
Dr. Shannon Toohey MD	LAB BLOOD ORDERABLES	Final Result

Performing Organization	Address	City/State/ZIP Code	Phone Number
UCI MEDICAL CENTER	101 The City Drive	Orange, CA 92868	

(ABNORMAL) COMPREHENSIVE METABOLIC PANEL, BLOOD - UCI - Final result (12/14/2025 11:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium	135 (L)	136 - 145 mmol/L		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Potassium	4.4	3.5 - 5.1 mmol/L		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Chloride	100	98 - 107 mmol/L		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Carbon Dioxide	28	21 - 31 mmol/L		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Comment: Samples with elevated Lactate Dehydrogenase (LDH) may cause falsely increased bicarbonate results. If elevated LDH levels are suspected, interpret bicarbonate results in the context of the patient's LDH values and overall clinical picture.						
Electrolyte Balance	7	2 - 12 mmol/L		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Glucose	112	70 - 115 mg/dL		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	

Comment:

Normal Fasting Glucose: <100 mg/dL

Impaired Fasting Glucose: 100-125 mg/dL

Provisional DX of diabetes (must be confirmed) >125 mg/dL

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
BUN	19	7 - 25 mg/dL		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Creatinine	1.0	0.7 - 1.3 mg/dL		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
eGFR	96	>=60 mL/min/1.73 sq mtr		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Comment: The reported eGFR is based on the CKD-EPI 2021 equation that does not use a race coefficient.						
The calculated GFR is an estimate and is NOT an accurate reflection of GFR in patients on dialysis. The accuracy of estimated GFR is also affected by muscle mass, and medications that affect the renal tubular secretion of creatinine.						
If estimated GFR will directly affect clinical decision-making, consider using cystatin C to estimate GFR, and/or consult with nephrology.						
Calcium	8.7	8.6 - 10.3 mg/dL		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Protein, Total	6.1	6.0 - 8.3 g/dL		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Albumin	4.3	4.2 - 5.5 g/dL		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Alkaline Phosphatase	49	34 - 104 U/L		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
AST	14	13 - 39 U/L		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
ALT	20	7 - 52 U/L		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Bilirubin, Total	0.4	<=1.4 mg/dL		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Specimen (Source)	Anatomical Location / Laterality		Collection Method / Volume	Collection Time		Received Time
Blood	Peripheral blood / Unknown		Venipuncture / Unknown	12/14/2025 11:45 AM PST		12/14/2025 11:54 AM PST
Narrative						
Authorizing Provider	Result Type		Result Status			
Dr. Shannon Toohey MD	LAB BLOOD ORDERABLES		Final Result			
Performing Organization	Address		City/State/ZIP Code	Phone Number		
UCI MEDICAL CENTER	101 The City Drive		Orange, CA 92868			

Care Teams				
Team Member	Relationship	Specialty	Start Date	End Date
Per Patient No PCP	PCP - General	Internal Medicine	12/14/25	

Patient Contacts

Contact Name	Contact Address	Communication	Relationship to Patient
Ann Verma	412 Olive Ave HUNTINGTON BEACH, CA 92648	949-350-2312 (Mobile) amd949609@gmail.com	Other, Emergency Contact

Document Information

Primary Care Provider	Other Service Providers	Document Coverage Dates
Per Patient No PCP (Dec. 14, 2025 - Present) Internal Medicine UCI Health 101 The City Drive South Orange, CA 92697		May 04, 1983 - Aug. 28, 2026
Custodian Organization		
UCI Health 101 The City Drive South Orange, CA 92697		
Legal Authenticator		
Background S		



If you take your Lucy record on a thumb drive to a different doctor, he or she might be able to use his computer to read the file electronically. Your downloaded, machine-readable Personal Health Summary document is in a format called "CDA." If your doctor has a computer that understands CDA, your information is a folder on your thumb drive called **MachineReadable_XDMFormat**. You might need to enter a password before your doctor can use this file.

Anthony Michael Dimarcello "Anthony"

Summary of Care, generated on Aug. 28, 2026

Patient Demographics - Male; born May 04, 1983

Patient Address	Patient Name	Communication
412 Olive Ave (Home) Huntington Beach, CA 92648-5142	Anthony Michael Dimarcello	949-350-2312 (Mobile) amd949609@gmail.com
Former (Aug. 17, 2026 - Aug. 17, 2026): 412 Olive Ave (Home) Huntington Beach, CA 92648	Former / Aliases: Anthony Dimarcello	
Former (Aug. 17, 2026 - Aug. 16, 2026): 412 Olive Ave (Home) Huntington Beach, B.C. 92648		
Former (Mar. 17, 2026 - Aug. 16, 2026): 412 Olive Ave (Home) Huntington Beach, CA 92648		
Former (Mar. 17, 2026 - Mar. 16, 2026): (Home)		
Language	Race / Ethnicity	Marital Status
English (Preferred)	White / Not Hispanic or Latino	Never Married

Note from UCI Health

This document contains information that was shared with Anthony Michael Dimarcello. It may not contain the entire record from UCI Health.

Reason for Visit

Reason
Syncope

Encounter Details

Date	Type	Department	Care Team (Latest Contact Info)	Description
12/14/2025 11:18 AM PST - 12/14/2025 5:31 PM PST	Emergency	ORA UCI HEALTH - ORANGE EMERGENCY DEPARTMENT 101 The City Drive South Orange, CA 92868-3201 714-456-5705	Dr. Shannon Toohey, MD 101 THE CITY DR ORANGE, CA 92868 714-456-7002 (Work) 714-456-6085 (Fax) Dr. Kristin Lewis, MD 101 THE CITY DR ORANGE, CA 92868 714-456-7002 (Work) 714-456-6085 (Fax)	Discharge Disposition: Home Routine

Allergies - documented as of this encounter (statuses as of 08/28/2026)

No known active allergies

Medications - documented as of this encounter (statuses as of 08/28/2026)

No known medications

Active Problems - documented as of this encounter (statuses as of 08/28/2026)

Not on file

Immunizations - documented as of this encounter

Immunization	Administration Dates	Next Due
Tdap	12/14/2025 (Deferred: Patient Refused - last Tdap <5 years)	

Social History - documented as of this encounter

Tobacco Use	Types	Packs/Day	Years Used	Date
Smoking Tobacco: Never				
Smokeless Tobacco: Current				
Tobacco Cessation: Ready to Quit: Not Asked; Counseling Given: Not Answered				
Alcohol Use	Standard Drinks/Week			
Not Currently	0 (1 standard drink = 0.6 oz pure alcohol)			
Sex and Gender Information		Value	Date Recorded	
Sex Assigned at Birth		Not on file		
Legal Sex		Male	12/14/2025 10:59 AM PST	
Gender Identity		Male	08/20/2026 12:01 PM PDT	
Sexual Orientation		Not on file		

Last Filed Vital Signs - documented in this encounter

Vital Sign	Reading	Time Taken	Comments
Blood Pressure	118/75	12/14/2025 4:45 PM PST	
Pulse	75	12/14/2025 4:45 PM PST	
Temperature	37 °C (98.6 °F)	12/14/2025 4:45 PM PST	
Respiratory Rate	16	12/14/2025 4:45 PM PST	
Oxygen Saturation	99%	12/14/2025 4:45 PM PST	
Inhaled Oxygen Concentration	-	-	
Weight	81.6 kg (180 lb)	12/14/2025 11:16 AM PST	
Height	180.3 cm (5' 11")	12/14/2025 11:16 AM PST	
Body Mass Index	25.1	12/14/2025 11:16 AM PST	

Functional Status - documented as of this encounter

Calculated C-SSRS Risk Score (Lifetime/Recent)			
Answer	Date of Assessment		Author
No Risk Indicated	12/14/2025 11:18 AM PST		Nurse Alexandra B
Columbia Suicide Severity Rating Scale (Screener/Recent Self-Report)			
Question	Answer	Date of Assessment	Author
1. Wish to be Dead (Past 1 Month)	No	12/14/2025 11:18 AM PST	Nurse Alexandra B
2. Non-Specific Active Suicidal Thoughts (Past 1 Month)	No	12/14/2025 11:18 AM PST	Nurse Alexandra B
6. Suicidal Behavior (Lifetime)	No	12/14/2025 11:18 AM PST	Nurse Alexandra B

Discharge Instructions - documented in this encounter

Discharge Instructions

Christine Hua, MD - 12/14/2025 12:24 PM PST

Formatting of this note might be different from the original.

Please have the staples in your scalp removed in 7-10 days (around 12/21)

You can have this done by your primary care doctor, or if not then at urgent care, and if not there then the emergency department

Diagnosis/Evaluation:

Today, you were seen for syncope (passing out) and head injury. We did CT scan, X-rays, Ultrasound, Lab tests, and EKG that showed no emergent findings. Based on your workup here in the emergency department, you were deemed stable for discharge.

Follow Up:

Please follow up with your primary care doctor in the next 2-5 days or sooner if needed

Medications:

Continue your home medications as previously prescribed.

Please return to the emergency department if you experience ANY of the following:

Severe worsening headache, persistent nausea/vomiting, vision changes, confusion, more episodes of passing out/losing consciousness

Fevers

Chest pain

Trouble breathing

Sudden weakness or numbness

ANY new or worsening symptoms that are concerning to you!

If you are in need of a primary care provider please see below:

UCI Health: Appointments - Family Health Center (FHC)

To schedule an initial or follow-up appointment with the FHC Anaheim offices, just call (657) 282-6356 and we will assist you through the process.

The FHC Anaheim offices are located at:

300 W Carl Karcher Way

Anaheim, CA 92801

The FHC Santa Ana medical offices are located at:

FHC Santa Ana Clinic

800 N. Main Street

Santa Ana, CA 92701

Please note: For UCI clinics and physicians other than the emergency department, your health insurance provider may or may not require prior authorization before an appointment at UCI can be made on your behalf. If you are not sure whether or not your health insurance company requires prior authorization, please call your health insurance provider first to prevent any unnecessary delays in your care.

Electronically signed by Christine Hua, MD at 12/14/2025 12:24 PM PST

Electronically signed by Christine Hua, MD at 12/14/2025 4:19 PM PST

Associated Order(s): Laceration Repair

Formatting of this note might be different from the original.

Procedure Note

Laceration Repair

Date/Time: 12/14/2025 12:20 PM

Performed by: Hua, Christine Tran, MD

Authorized by: Hua, Christine Tran, MD

Consent Obtained: Verbal

Consent given by: Patient

Risks and benefits: Risks, benefits and alternatives were discussed

Indication: Laceration

Consent: Prior to the procedure, the risks, benefits and alternatives were explained to the patient. The patient gave verbal consent to the procedure.

Timeout: Prior to the procedure, a time-out was called and correct patient and procedure were confirmed.

Site: left occipital scalp

Procedure: Prior to the repair, the patient's wound was cleansed. A total of 50 ml of normal saline was used to thoroughly irrigate the wound under pressure. The wound was inspected and no obvious foreign objects were found. The patient's wound was repaired with 6 staples. Total wound length was 3 cm. The patient tolerated the procedure well and there were no immediate complications.

Supervised Procedure: I am a resident/fellow and an attending was present to supervise the procedure patient tolerated the procedure well with no immediate complications

Hua, Christine Tran, MD

Resident

12/14/25 2009

Shannon L Toohey

Toohey, Shannon L, MD

12/15/25 1308

Electronically signed by Christine Hua, MD at 12/14/2025 8:09 PM PST

Electronically signed by Dr. Shannon Toohey, MD at 12/15/2025 1:08 PM PST

Formatting of this note is different from the original.

CHIEF COMPLAINT:

Chief Complaint

Patient presents with

Syncope

BIBA from Theo Lacy s/p syncopal episode. +laceration to occipital area.

HISTORY OF PRESENT ILLNESS:

Interpreter used: No (English Preferred Language)

Anthony Michael Dimarcello is a 42 years old male with no significant pmhx brought in by ambulance from Theo Lacy s/p syncope and head injury.

Patient states he was urinating when he had sudden onset dizziness and then syncopized, falling and hitting his posterior head on the ground. Per EMS: patient hypotensive on scene, BP 80s/60s, which improved on route after administration of IV fluids. Patient currently denies any symptoms. Denies chest pain, shortness of breath, cough, fever, abdominal pain, nausea/vomiting, dizziness, paresthesias, or headache. Not on anticoagulants.

PMH/SHx/Allgs/FHx/Shx/SDOH:

History reviewed. No pertinent past medical history.
There are no active problems to display for this patient.

History reviewed. No pertinent surgical history.
No Known Allergies
Reviewed and considered non-contributory
none

Socioeconomic History
Marital status: Never Married
Tobacco Use
Smoking status: Never
Smokeless tobacco: Current
Substance and Sexual Activity
Alcohol use: Not Currently
Drug use: Not Currently

VITAL SIGNS:
First Vitals [12/14/25 1116]
Temp Pulse Resp BP SpO2
36.7 °C (98 °F) 62 16 116/79 99 %

Physical Exam
General: Awake, alert, in no apparent distress.
HEENT: Normocephalic, +3cm laceration to left occipital scalp. PERRL, EOMI, moist mucous membranes.
Neck: Negative C-spine ttp, c-collar in place.
Respiratory: Lungs CTAB, no wheezing, no crackles. Breathing comfortably and in no respiratory distress.
Cardiovascular: RRR, no obvious murmurs. Extremities are warm and well perfused.
Abdomen: Soft, nontender, nondistended, no rebound/guarding.
Back: Negative midline T/L-spine ttp.
Extremities: No LE edema, clubbing, or cyanosis. No tenderness to joints or extremities.
Neuro: Awake and alert. Moving extremities. No focal neuro deficits. Clear speech.
Psych: Answers questions appropriately.

MEDICAL DECISION MAKING

Initial Impressions:
Anthony Michael Dimarcello is a 42 years old male who presents with syncope and fall with head injury.

Vital signs and physical exam were notable for: Afebrile, nontachycardic, normotensive, saturating appropriately on room air. Exam with 3cm occipital scalp laceration, no focal neuro deficits, rest of exam unremarkable.

Differential diagnosis includes: vasovagal syncope, orthostatics given hypotension reported in field, dehydration, electrolyte derangement, lower suspicion for ICH, exam and history not consistent with seizure given no seizure-like activity no postictal state, low suspicion for acute underlying infection given no fever no cough no reported infectious symptoms

Workup Review:
Pertinent Lab Results (interpreted independently by me): CBC with no leukocytosis doubt severe systemic infection, no anemia doubt severe hemorrhage as cause of symptoms, no thrombocytopenia. CMP with no major electrolyte derangements, no AKI. Serial troponins negative x3.

Pertinent Imaging Results (interpreted independently by me):
- CT head negative for acute ICH or skull fracture
- CT C-spine negative for acute fracture
- Chest XR negative for obvious focal consolidation, pleural effusion, or large pneumothorax per my read

EKG (interpreted independently by me): sinus rhythm at 62 bpm, normal axis, normal intervals, no evidence of acute ischemic changes

Patient here after syncopal episode with collapse and head injury. Had preceding symptoms of dizziness, no other chest pain or shortness of breath. Was noted to be hypotensive in the field, which rapidly improved on reassessment and after fluid bolus and has remained stable. On arrival to ED, patient awake alert mentating appropriately with no focal neuro deficits. Small occipital scalp laceration cleaned and repaired with staples x6, patient tolerated procedure well. CT imaging negative for any acute traumatic findings. Labs overall unremarkable. TEE obtained with LV EF 52%, no other abnormal findings. Serial troponins negative x3 and nonischemic EKG, doubt ACS. Patient observed for quite some time in the emergency department with no further episodes of syncope or acute decompensation. Patient ambulated and tolerated PO. Discussed with him all results and plan for close PCP follow-up. Strict return precautions provided. He verbalized understanding and was amenable with plan of care, hemodynamically stable and symptomatically improved upon discharge back to facility.

Disposition Decision:
Discharge

Patient's exam and workup were reassuring. They are stable for discharge at this time and I recommend that they pursue outpatient follow up for their complaint. Questions answered and return precautions provided.

There are no discharge medications for this patient.

The following work up was performed during the encounter. Orders placed after a disposition has been selected will not be listed here. A complete account of orders should be referenced elsewhere:

ED Orders (720h ago, onward)

Start Ordered Status Ordering Provider

12/14/25 1412 12/14/25 1411 Troponin I, High Sensitivity Once

Final result TOOHEY, SHANNON L

12/14/25 1221 12/14/25 1220 Laceration Repair Once

Comments: This order was created via procedure documentation

Preliminary result HUA, CHRISTINE TRAN

12/14/25 1156 12/14/25 1155 Extra Tubes Once

Final result TOOHEY, SHANNON L

12/14/25 1156 12/14/25 1155 Extra Gray PROCEDURE ONCE

Final result TOOHEY, SHANNON L

12/14/25 1145 12/14/25 1136 sodium chloride 0.9 % 500 mL IV bolus ONCE

Last MAR action: New Bag HUA, CHRISTINE TRAN

12/14/25 1145 12/14/25 1136 acetaminophen (Ofirmev) IV injection 1,000 mg ONCE

Last MAR action: New Bag HUA, CHRISTINE TRAN

12/14/25 1138 12/14/25 1138 CT Head W/O Contrast Once

Final result HUA, CHRISTINE TRAN

12/14/25 1138 12/14/25 1138 CT C-Spine W/O Contrast Once

Final result HUA, CHRISTINE TRAN

12/14/25 1137 12/14/25 1136 Troponin I, High Sensitivity Every 1 hour

Order ID Start Status Ordering Provider

851231260 12/14/25 1137 Final result HUA, CHRISTINE TRAN

851231261 12/14/25 1200 Final result HUA, CHRISTINE TRAN

Completed HUA, CHRISTINE TRAN

12/14/25 1136 12/14/25 1136 Insert peripheral IV Once

Completed HUA, CHRISTINE TRAN

12/14/25 1136 12/14/25 1136 TRANSTHORACIC ECHO (TTE) Once

Final result HUA, CHRISTINE TRAN

12/14/25 1135 12/14/25 1136 CBC w/ Diff Once

Final result HUA, CHRISTINE TRAN

12/14/25 1135 12/14/25 1136 PT/INR/PTT Once

Final result HUA, CHRISTINE TRAN

12/14/25 1135 12/14/25 1136 Comprehensive Metabolic Panel Once

Final result HUA, CHRISTINE TRAN

12/14/25 1135 12/14/25 1136 ECG 12-LEAD Once

Preliminary result HUA, CHRISTINE TRAN

12/14/25 1135 12/14/25 1136 Hemogram PROCEDURE ONCE

Final result HUA, CHRISTINE TRAN

12/14/25 1135 12/14/25 1136 Differential PROCEDURE ONCE

Final result HUA, CHRISTINE TRAN

12/14/25 1135 12/14/25 1136 X-Ray Chest Single View Once

Final result HUA, CHRISTINE TRAN

ED COURSE/PATIENT REASSESSMENTS:

"ED Course" is listed below that provides real time documentation during ER encounter and will provide further context to the MDM discussion above.

ED Course as of 12/14/25 2038

Sun Dec 14, 2025

1256 CBC within normal limits, no anemia or leukocytosis. CMP with no clinically significant abnormalities; no evidence of electrolyte abnormalities. Trop <3 doubt ACS [ST]

1348 ECG with no ST elevations or depressions to suggest acute ischemia; no arrhythmia to suggest emergent cardiac cause of

symptoms [ST]

1409 CT head without ICH on my read [ST]

1410 Sign out from ST to KL

Illness severity: Stable

Patient summary: 42yo M with syncope at jail. Labs so far unremarkable

Action list: CT reads, 2nd trop, echo

Situation awareness: likely dc if nothing on echo [ST]

1505 CT Head W/O Contrast

IMPRESSION:

No acute intracranial hemorrhage, midline shift, herniation, or hydrocephalus.

Scalp injury without underlying acute fracture.

No acute cervical spine fracture.

[CH]

1540 X-Ray Chest Single View

Frontal view chest. Clear lungs. No pleural effusion or pneumothorax. Normal cardiomedastinal silhouette.

No acute osseous abnormality.

[CH]

1601 Trop neg x 3 [KL]

1610 On reassessment: patient sitting upright in stretcher, tolerating PO, denies any symptoms, states feeling overall improved.

[CH]

1615 TRANSTHORACIC ECHO (TTE)

1. Low normal left ventricular systolic function. The left ventricular ejection fraction is 52% by 3D

2. Normal diastolic LV function.

3. Normal LV filling pressures.

4. Normal right ventricular size and systolic function. [KL]

ED Course User Index

[CH] Hua, Christine Tran, MD

[KL] Lewis, Kristin M, MD

[ST] Toohey, Shannon L, MD

Diagnoses as of 12/14/25 2038

Syncope and collapse

Laceration of occipital scalp, initial encounter

Closed head injury, initial encounter

DIAGNOSIS:

1. Syncope and collapse

2. Laceration of occipital scalp, initial encounter

3. Closed head injury, initial encounter

Hua, Christine Tran, MD

Resident

12/14/25 2039

ATTENDING ATTESTATION:

I evaluated the patient concurrently with the Resident/Fellow. I discussed the case with the Resident/Fellow and agree with the findings and plan as documented by the Resident/Fellow. Any additions or revisions are included in the record as necessary.

Shannon L Toohey, MD

Toohey, Shannon L, MD

12/15/25 1308

Electronically signed by Christine Hua, MD at 12/14/2025 8:39 PM PST

Electronically signed by Dr. Shannon Toohey, MD at 12/15/2025 1:08 PM PST

Miscellaneous Notes - documented in this encounter

Flowsheet Note - Nurse Hung N - 12/14/2025 2:48 PM PST

Formatting of this note is different from the original.

12/14/25 1448
NEXUS Criteria (If YES to any of the NEXUS criteria, do not complete the ROM assessment and maintain cervical collar.
Is the patient altered, lethargic, obtunded, stuporous, comatose or intubated? No
Does the patient appear to be intoxicated based on physical exam? No
Is the patent GCS 14 or less, disorientated to person, place, time or event, delayed in response or having any focal deficit on motor or sensory examination? No
Is there any midline tenderness on palpation from the base of the skull to the prominence of the first thoracic vertebrae (T1)? No
Does the patient have any distracting injuries, such as a long bone fracture, large burn, or any injuries in which the pain is so severe that the patient cannot participate in the exam? No
Range of Motion Assessment
Midline Pain on Flexion No
Midline Pain on Extension No
Midline Pain on Rotation No
(If YES to any of the findings above, maintain c-collar.)
C-Collar Intervention Removed C-collar

Electronically signed by Nurse Hung N at 12/14/2025 2:49 PM PST

Plan of Treatment - documented as of this encounter

Health Maintenance	Due Date	Last Done	Comments
HPV (Human Papillomavirus) Vaccine (1 - Male 3-dose series)	05/04/1998		
DTaP/Tdap/Td Vaccines (1 - Tdap)	05/04/2002		
Influenza Vaccine (#1)	09/01/2026		
Diabetes Screening	12/14/2028	12/14/2025	
Zoster (Shingles) Vaccine (1 of 2)	05/04/2033		
Meningitis Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Pneumococcal Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic
Rotavirus Vaccine	Aged Out		No longer eligible based on patient's age to complete this topic

Procedures - documented in this encounter

Procedure Name	Priority	Date/Time	Associated Diagnosis	Comments
TRANSTHORACIC ECHO (TTE)	STAT	12/14/2025 4:00 PM PST		
LACERATION REPAIR	Routine	12/14/2025 12:20 PM PST		

Results - documented in this encounter

TRANSTHORACIC ECHO (TTE) - Final result (12/14/2025 4:00 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
LV Stroke Volume Index	37.8	ml/m2			CPACS	
AO Peak Velocity	1.34	m/s			CPACS	
LV Diastolic Diameter	5.36	cm			CPACS	
Mitral Valve Area	4.0	mmHg			CPACS	
Aortic Valve Area	1.98	cm2			CPACS	
Mitral Valve Area	3.50	cm2			CPACS	
LV Ejection Fraction	62.1	%			CPACS	
LA Volume Index	29.2	ml/m2			CPACS	

Anatomical Region	Laterality	Modality
Cardiac/Heart		Echography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/14/2025 3:29 PM PST	

Narrative

12/14/2025 4:14 PM PST

UCI Health
Cardiac Diagnostic Center Echocardiography Report

Name: ANTHONY MICHAEL DIMARCELLO Sex: M Date/Time: 12/14/2025 3:29:50 PM
MRN #: 5405781 DOB: 5/4/1983 Age: 42 years
ACC #: 251317823 Ht: 180.30 cm BP: 110 / 78 mmHg
Location: IP ED STAT Wt: 81.65 kg BSA: 2.02 m² Heart Rate: 72 bpm
Billing Location Orange 80102067.

Referring Physician: 112276 SHANNON L TOOHEY , cc:
Indications: Syncope
Procedure Codes: Complete echo without contrast with spectral strain and 3D
- 4839330602;- 4809335600;- 3507637601;- 93306;- 93356;- 76376
Technical Quality: Image quality for this study is adequate.

Appropriate Use Criteria: Symptoms or conditions potentially related to suspected cardiology etiology;
AUC score = 9.

2D AND M-MODE MEASUREMENTS (normal ranges within parentheses):
Left Ventricle: Normal Aorta/Left Atrium: Normal
IVSd (2D): 1.11 cm (0.7-1.1) Aortic Sinus: 3.53 cm
LVPWd (2D): 0.97 cm (0.7-1.1) Left Atrium (2D): 2.60 cm (1.9-4.0)
LVIDd (2D): 5.36 cm (3.4-5.7) LA vol Index A/L:
LVIDs (2D): 3.95 cm LA vol Index A4C 19.6 ml/mA²
LVEDV Index 60.79 LA vol Index A2C 34.9 ml/mA²
LA vol Index BP 29.2 ml/mA²
Right Ventricle: RA vol Index A/L:
Rvd A4C base(2D): 3.16 cm RA vol Index A4C 24.0 ml/mA²

LV SYSTOLIC FUNCTION BY 2D PLANIMETRY (MOD):
EF-Biplane: 62.1 %
LV stroke volume index-Biplane: 37.8 ml/mA²
LV GLS: -19.3 %

LV DIASTOLIC FUNCTION:
MV Peak E: 0.57 m/s e', MV Ann: 0.12 m/s MV A Dur 116 msec
MV Peak A: 0.46 m/s E/e' Ratio: 4.56
E/A Ratio: 1.25 Decel Time: 217 msec

SPECTRAL DOPPLER ANALYSIS (where applicable)
MV P1/2 Time: 62.93 msec
MV Area, PHT: 3.50 cmA²
MR VTI:

Narrative

Right Ventricle (RV) Function by 2D: RV TAPSE: 2.1 cm

Aortic Valve: AoV Max Velocity: 1.34 m/s AoV Peak PG: 7.2 mmHg AoV Mean PG: 4.0 mmHg

LVOT Vmax: 0.91 m/s LVOT VTI: 0.150 m LVOT Diameter: 1.90 cm

AoV Area, Vmax: 1.93 cmA² AoV Area, VTI: 1.98 cmA² AoV Area, Vmn: 2.05 cmA²
AV AT: AVAI: 0.98 cmA²/mA² DVI:

Tricuspid Valve and PA/RV Systolic Pressure: TR Max Velocity: RA Pressure: 3 mmHg RVSP/PASP:

Pulmonic Valve:

PV Max Velocity: 1.02 m/s PV Max PG: 4.2 mmHg PV Mean PG:

PHYSICIAN INTERPRETATION:

Left Ventricle: The left ventricular internal size is normal. No evidence of left ventricular hypertrophy. There is global Low normal left ventricular systolic function. The left ventricular ejection fraction is 52% by 3D. Left ventricular ejection fraction is normal diastolic left ventricular function is noted. Normal LV filling pressures. Global longitudinal strain is normal.

LV wall Scoring:

All segments are normal.

Right Ventricle: Normal right ventricular size and systolic function.

Left Atrium: The left atrium is normal in size and structure.

Right Atrium: The right atrium is normal in size and structure.

Mitral Valve: Structurally normal mitral valve, with normal leaflet excursion. No evidence of mitral valve regurgitation is seen.

Tricuspid Valve: The tricuspid valve is normal in structure. No tricuspid regurgitation was seen.

Aortic Valve: The aortic valve is tricuspid. The aortic valve is without evidence of sclerosis or stenosis. No evidence of aortic valve regurgitation is seen.

Pulmonic Valve: The pulmonic valve was not well visualized. No indication of pulmonic valve regurgitation. No evidence of pulmonic stenosis.

Aorta: The aortic root is normal in size and structure.

Pericardium: There is no evidence of pericardial effusion.

Pulmonary Artery: Inadequate Doppler signal to assess pulmonary artery systolic pressure.

Venous: A normal flow pattern is recorded from the right upper pulmonary vein. The inferior vena cava is normal in size and exhibits greater than 50% respiratory size variation.

Summary:

1. Low normal left ventricular systolic function. The left ventricular ejection fraction is 52% by 3D
2. Normal diastolic LV function.
3. Normal LV filling pressures.
4. Normal right ventricular size and systolic function.

In comparison to the previous echocardiogram(s): There are no prior studies on this patient for comparison purposes.

122986 Behram Mody MD Sonographer: BT
UCI Division of Cardiology

Electronically signed by 122986 Behram Mody MD on 12/14/2025 at 4:14:30 PM
ICD CODES: Syncope

Final

Procedure Note

Dr. Behram Mody, MD - 12/14/2025

Formatting of this note might be different from the original.

UCI Health

Cardiac Diagnostic Center Echocardiography Report

Name: ANTHONY MICHAEL Sex: M Date/Time: 12/14/2025 3:29:50

DIMARCELLO PM

MRN #: 5405781 DOB: 5/4/1983 Age: 42 years

ACC #: 251317823 Ht: 180.30 BP: 110 / 78 mmHg

cm

Location: IP ED STAT Wt: 81.65 kg BSA: 2.02 mA² Heart Rate: 72

bpm

Billing Location Orange 80102067.

Referring Physician: 112276 SHANNON L TOOHEY , cc:

Indications: Syncope

Procedure Codes: Complete echo without contrast with spectral strain and 3D

- 4839330602;- 4809335600;- 3507637601;- 93306;- 93356;- 76376

Technical Quality: Image quality for this study is adequate.

Appropriate Use Criteria: Symptoms or conditions potentially related to suspected cardiology etiology; AUC score = 9.

2D AND M-MODE MEASUREMENTS (normal ranges within parentheses):

Left Ventricle: Normal Aorta/Left Atrium: Normal

IVSd (2D): 1.11 cm (0.7-1.1) Aortic Sinus: 3.53 cm

LVPWd (2D): 0.97 cm (0.7-1.1) Left Atrium (2D): 2.60 cm (1.9-4.0)

LVIDd (2D): 5.36 cm (3.4-5.7) LA Vol Index A/L:

LVIDs (2D): 3.95 cm LA Vol Index A4C 19.6 ml/mA²

LVEDV Index 60.79 LA Vol Index A2C 34.9 ml/mA²

LA Vol Index BP 29.2 ml/mA²

Right Ventricle: RA Vol Index A/L:

RVd A4C base(2D): 3.16 cm RA Vol Index A4C 24.0 ml/mA²

LV SYSTOLIC FUNCTION BY 2D PLANIMETRY (MOD):

EF-Biplane: 62.1 %

LV stroke volume index-Biplane: 37.8 ml/mA²

LV GLS: -19.3 %

LV DIASTOLIC FUNCTION:

MV Peak E: 0.57 m/s e', MV Ann: 0.12 m/s MV A Dur 116 msec

MV Peak A: 0.46 m/s E/e' Ratio: 4.56

E/A Ratio: 1.25 Decel Time: 217 msec

SPECTRAL DOPPLER ANALYSIS (where applicable)

MV P1/2 Time: 62.93 msec

MV Area, PHT: 3.50 cmA²

MR VTI:

Right Ventricle (RV) Function by 2D: RV TAPSE: 2.1 cm

Aortic Valve: AoV Max Velocity: 1.34 m/s AoV Peak PG: 7.2 mmHg AoV Mean PG: 4.0 mmHg

LVOT Vmax: 0.91 m/s LVOT VTI: 0.150 m LVOT Diameter: 1.90 cm

AoV Area, Vmax: 1.93 cmA² AoV Area, VTI: 1.98 cmA² AoV Area, Vmn: 2.05 cmA²

AV AT: AVAI: 0.98 cmA²/mA² DVI:

Tricuspid Valve and PA/RV Systolic Pressure: TR Max Velocity: RA Pressure: 3 mmHg RVSP/PASP:

Pulmonic Valve:

PV Max Velocity: 1.02 m/s PV Max PG: 4.2 mmHg PV Mean PG:

PHYSICIAN INTERPRETATION:

Left Ventricle: The left ventricular internal size is normal. No evidence of left ventricular hypertrophy. There is global Low normal left ventricular systolic function. The left ventricular ejection fraction is 52% by 3D. Left ventricular ejection fraction is normal diastolic left ventricular function is noted. Normal LV filling pressures. Global longitudinal strain is normal.

LV Wall Scoring:

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Right Ventricle: Normal right ventricular size and systolic function.

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Right Atrium: The right atrium is normal in size and structure.

Mitral Valve: Structurally normal mitral valve, with normal leaflet excursion. No evidence of mitral valve regurgitation is seen.

Tricuspid Valve: The tricuspid valve is normal in structure. No tricuspid regurgitation was seen.

Procedure Note

Aortic Valve: The aortic valve is tricuspid. The aortic valve is without evidence of sclerosis or stenosis. No evidence of aortic valve regurgitation is seen.

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Pulmonary Artery: Inadequate Doppler signal to assess pulmonary artery systolic pressure.

Venous: A normal flow pattern is recorded from the right upper pulmonary vein. The inferior vena cava is normal in size and exhibits greater than 50% respiratory size variation.

Summary:

- 1. Low normal left ventricular systolic function.The left ventricular ejection fraction is 52% by 3D
- 2. Normal diastolic LV function.
- 3. Normal LV filling pressures.
- 4. Normal right ventricular size and systolic function.

In comparison to the previous echocardiogram(s): There are no prior studies on this patient for comparison purposes.

122986 Behram Mody MD Sonographer: BT
UCI Division of Cardiology

Electronically signed by 122986 Behram Mody MD on 12/14/2025 at 4:14:30 PM
ICD CODES: Syncope

Final

Authorizing Provider	Result Type	Result Status
Dr. Shannon Toohey MD	CV ECHO PROCEDURES	Final Result

CT C-Spine W/O Contrast (CT C-SPINE W/O CONTRAST) - Final result (12/14/2025 2:54 PM PST)

Anatomical Region	Laterality	Modality
Spine, C-spine, Musculoskeletal		Computed Tomography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/14/2025 2:23 PM PST	

Impressions

12/14/2025 2:36 PM PST

No acute intracranial hemorrhage, midline shift, herniation, or hydrocephalus.

Scalp injury without underlying acute fracture.

No acute cervical spine fracture.

I have personally reviewed the images upon which this report is based and agree with the findings and conclusions expressed above.

Narrative

12/14/2025 2:36 PM PST

CT HEAD W/O CONTRAST, CT C-SPINE W/O CONTRAST

INDICATION: SYNCOPE, head trauma

EXAM DATE: 12/14/2025 2:19 PM

COMPARISON: None

TECHNIQUE:

CT head without intravenous contrast.

CT cervical spine without intravenous contrast.

RADIATION DOSE:

This patient received a total of 2 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 203; Anatomy: C Spine; Phantom: 32 cm; CTDIvol: 11; DLP: 298

Exposure: 2; Series: 303; Anatomy: C Spine; Phantom: 16 cm; CTDIvol: 46; DLP: 1021

The following accession numbers are related to this dose report {251317997}:

251318064

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient. CT scan was completed using equipment that meets the NEMA XR-29 safety standards.

(accession 251317997),

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Exposure: 2; Series: 303; Anatomy: C Spine; Phantom: 16 cm; CTDIvol: 46; DLP: 1021

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(accession 251318064)

FINDINGS:

CT head:

Hemorrhage: None.

Brain parenchyma: Unremarkable.

Mass effect: No midline shift. No herniation. No hydrocephalus.

Osseous/extracranial soft tissues: No acute calvarial fracture. Left posterior scalp injury with skin staples.

CT cervical spine:

Alignment: Straightening of the usual cervical lordosis.

vertebral bodies and posterior elements: No acute fracture. Intact heights. No suspicious lesion.

Discs and joints: Mild degenerative changes.

Cranio cervical junction: Intact.

Soft tissues: No prevertebral edema. Intact surrounding soft tissues.

Additional findings: None.

Formatting of this note might be different from the original.

CT HEAD W/O CONTRAST, CT C-SPINE W/O CONTRAST

INDICATION: SYNCOPE, head trauma

EXAM DATE: 12/14/2025 2:19 PM

COMPARISON: None

TECHNIQUE:

CT head without intravenous contrast.

CT cervical spine without intravenous contrast.

RADIATION DOSE:

This patient received a total of 2 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 203; Anatomy: C Spine; Phantom: 32 cm; CTDIvol: 11; DLP: 298

Exposure: 2; Series: 303; Anatomy: C Spine; Phantom: 16 cm; CTDIvol: 46; DLP: 1021

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(accession 251318064)

FINDINGS:

CT head:

Hemorrhage: None.

Brain parenchyma: Unremarkable.

Mass effect: No midline shift. No herniation. No hydrocephalus.

Osseous/extracranial soft tissues: No acute calvarial fracture. Left posterior scalp injury with skin staples.

CT cervical spine:

Alignment: Straightening of the usual cervical lordosis.

Vertebral bodies and posterior elements: No acute fracture. Intact heights. No suspicious lesion.

Discs and joints: Mild degenerative changes.

Procedure Note

Craniocervical junction: Intact.

Soft tissues: No prevertebral edema. Intact surrounding soft tissues.

Additional findings: None.

IMPRESSION:

No acute intracranial hemorrhage, midline shift, herniation, or hydrocephalus.

Scalp injury without underlying acute fracture.

No acute cervical spine fracture.

I have personally reviewed the images upon which this report is based and agree with the findings and conclusions expressed above.

Authorizing Provider	Result Type	Result Status
Dr. Shannon Toohey MD	IMG CT PROCEDURES	Final Result

CT Head W/O Contrast (CT HEAD W/O CONTRAST) - Final result (12/14/2025 2:54 PM PST)

Anatomical Region	Laterality	Modality
Head and Neck		Computed Tomography

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/14/2025 2:23 PM PST	

Impressions

12/14/2025 2:36 PM PST

No acute intracranial hemorrhage, midline shift, herniation, or hydrocephalus.

Scalp injury without underlying acute fracture.

No acute cervical spine fracture.

I have personally reviewed the images upon which this report is based and agree with the findings and conclusions expressed above.

Narrative

12/14/2025 2:36 PM PST

CT HEAD W/O CONTRAST, CT C-SPINE W/O CONTRAST

INDICATION: SYNCOPE, head trauma

EXAM DATE: 12/14/2025 2:19 PM

COMPARISON: None

TECHNIQUE:

CT head without intravenous contrast.

CT cervical spine without intravenous contrast.

RADIATION DOSE:

This patient received a total of 2 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 203; Anatomy: C Spine; Phantom: 32 cm; CTDIvol: 11; DLP: 298

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The following accession numbers are related to this dose report {251317997}:

251318064

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient. CT scan was completed using equipment that meets the NEMA XR-29 safety standards.

(accession 251317997),

This patient received a total of 2 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 2; Series: 303; Anatomy: C Spine; Phantom: 16 cm; CTDIvol: 46; DLP: 1021

Exposure: 1; Series: 203; Anatomy: C Spine; Phantom: 32 cm; CTDIvol: 11; DLP: 298

The following accession numbers are related to this dose report {251318064}:

251317997

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient. CT scan was completed using equipment that meets the NEMA XR-29 safety standards.

(accession 251318064)

FINDINGS:

CT head:

Hemorrhage: None.

Brain parenchyma: Unremarkable.

Mass effect: No midline shift. No herniation. No hydrocephalus.

Osseous/extracranial soft tissues: No acute calvarial fracture. Left posterior scalp injury with skin staples.

CT cervical spine:

Alignment: Straightening of the usual cervical lordosis.

vertebral bodies and posterior elements: No acute fracture. Intact heights. No suspicious lesion.

Discs and joints: Mild degenerative changes.

Cranio cervical junction: Intact.

Soft tissues: No prevertebral edema. Intact surrounding soft tissues.

Additional findings: None.

Formatting of this note might be different from the original.

CT HEAD W/O CONTRAST, CT C-SPINE W/O CONTRAST

INDICATION: SYNCOPE, head trauma

EXAM DATE: 12/14/2025 2:19 PM

COMPARISON: None

TECHNIQUE:

CT head without intravenous contrast.

CT cervical spine without intravenous contrast.

RADIATION DOSE:

This patient received a total of 2 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 1; Series: 203; Anatomy: C Spine; Phantom: 32 cm; CTDIvol: 11; DLP: 298

Exposure: 2; Series: 303; Anatomy: C Spine; Phantom: 16 cm; CTDIvol: 46; DLP: 1021

The following accession numbers are related to this dose report {251317997}:

251318064

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient. CT scan was completed using equipment that meets the NEMA XR-29 safety standards.

(accession 251317997),

This patient received a total of 2 exposure event(s) during this CT examination. The CTDIvol and DLP radiation dose values for each exposure are:

Exposure: 2; Series: 303; Anatomy: C Spine; Phantom: 16 cm; CTDIvol: 46; DLP: 1021

Exposure: 1; Series: 203; Anatomy: C Spine; Phantom: 32 cm; CTDIvol: 11; DLP: 298

The following accession numbers are related to this dose report {251318064}:

251317997

The dose indicators for CT are the volume Computed Tomography (CT) Dose Index (CTDIvol) and the Dose Length Product (DLP), and are measured in units of mGy and mGy-cm, respectively. These indicators are not patient dose, but values generated from the CT scanner acquisition factors. The report includes radiation exposure data for exposures received during this examination. If multiple reports are produced from this examination, the exposure data is duplicated in each report. The exposure data reported is indicative, but not determinative, of the radiation dose received by this patient. CT scan was completed using equipment that meets the NEMA XR-29 safety standards.

(accession 251318064)

FINDINGS:

CT head:

Hemorrhage: None.

Brain parenchyma: Unremarkable.

Mass effect: No midline shift. No herniation. No hydrocephalus.

Osseous/extracranial soft tissues: No acute calvarial fracture. Left posterior scalp injury with skin staples.

CT cervical spine:

Alignment: Straightening of the usual cervical lordosis.

Vertebral bodies and posterior elements: No acute fracture. Intact heights. No suspicious lesion.

Discs and joints: Mild degenerative changes.

Procedure Note

Craniocervical junction: Intact.

Soft tissues: No prevertebral edema. Intact surrounding soft tissues.

Additional findings: None.

IMPRESSION:

No acute intracranial hemorrhage, midline shift, herniation, or hydrocephalus.

Scalp injury without underlying acute fracture.

No acute cervical spine fracture.

I have personally reviewed the images upon which this report is based and agree with the findings and conclusions expressed above.

Authorizing Provider	Result Type	Result Status
Dr. Shannon Toohey MD	IMG CT PROCEDURES	Final Result

TROPONIN I, HIGH SENSITIVITY (ED) - UCI - Final result (12/14/2025 2:16 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I, High Sensitivity	<3	0 - 20 ng/L		12/14/2025 3:04 PM PST	UCI MEDICAL CENTER	
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time		
Blood	Peripheral blood / Unknown	Venipuncture / Unknown	12/14/2025 2:16 PM PST	12/14/2025 2:35 PM PST		

Narrative

UCI MEDICAL CENTER - 12/14/2025 3:04 PM PST
These values were determined with a high sensitivity troponin I assay. The 99th percentile upper reference limits established by the manufacturer are:

Females: 15 ng/L
Males: 20 ng/L
Overall: 18 ng/L

Authorizing Provider	Result Type	Result Status	
Dr. Shannon Toohey MD	LAB BLOOD ORDERABLES	Final Result	
Performing Organization	Address	City/State/ZIP Code	Phone Number
UCI MEDICAL CENTER	101 The City Drive	Orange, CA 92868	

ECG 12-LEAD - Edited Result - FINAL (12/14/2025 1:44 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Heart Rate	62	bpm			FOUNDATION RADIOLOGY SYSTEM	
RR Interval	968	ms			FOUNDATION RADIOLOGY SYSTEM	
Atrial Rate	63	ms			FOUNDATION RADIOLOGY SYSTEM	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
PR Interval	157	ms			FOUNDATION RADIOLOGY SYSTEM	
P Front Axis	63	deg			FOUNDATION RADIOLOGY SYSTEM	
QRSD Interval	93	ms			FOUNDATION RADIOLOGY SYSTEM	
QT Interval	456	ms			FOUNDATION RADIOLOGY SYSTEM	
QTcF	461	ms			FOUNDATION RADIOLOGY SYSTEM	
QTcFm	460	ms			FOUNDATION RADIOLOGY SYSTEM	
QRS Axis	60	deg			FOUNDATION RADIOLOGY SYSTEM	
T Wave Axis	81	deg			FOUNDATION RADIOLOGY SYSTEM	
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume		Collection Time	Received Time	
				12/14/2025 1:43 PM PST		

Narrative

FOUNDATION RADIOLOGY SYSTEM - 12/15/2025 10:16 PM PST
This result has an attachment that is not available.
- BORDERLINE ECG -
Sinus rhythm
Borderline T wave abnormalities
No previous ECG available for comparison

Procedure Note

Dr. Satinder Swaroop, MD - 01/20/2026
Formatting of this note might be different from the original.
- BORDERLINE ECG -
Sinus rhythm
Borderline T wave abnormalities
No previous ECG available for comparison

Authorizing Provider	Result Type	Result Status	
Dr. Shannon Toohey MD	ECG ORDERABLES	Edited Result - Final	
Performing Organization	Address	City/State/ZIP Code	Phone Number
FOUNDATION RADIOLOGY SYSTEM	123 Anywhere Street	Madison, WI 53593, US	

TROPONIN I, HIGH SENSITIVITY (ED) - UCI - Final result (12/14/2025 12:44 PM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I, High Sensitivity	<3	0 - 20 ng/L		12/14/2025 1:40 PM PST	UCI MEDICAL CENTER	
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume		Collection Time	Received Time	
Blood	Peripheral blood / Unknown	Venipuncture / Unknown		12/14/2025 12:44 PM PST	12/14/2025 1:10 PM PST	

Narrative

UCI MEDICAL CENTER - 12/14/2025 1:40 PM PST

These values were determined with a high sensitivity troponin I assay. The 99th percentile upper reference limits established by the manufacturer are:

Females: 15 ng/L
Males: 20 ng/L
Overall: 18 ng/L

Authorizing Provider	Result Type	Result Status	
Dr. Shannon Toohey MD	LAB BLOOD ORDERABLES	Final Result	
Performing Organization	Address	City/State/ZIP Code	Phone Number
UCI MEDICAL CENTER	101 The City Drive	Orange, CA 92868	

LACERATION REPAIR (Laceration Repair) - Final result (12/14/2025 12:20 PM PST)

Narrative

Dr. Shannon Toohey, MD - 12/14/2025 12:20 PM PST

Toohey, Shannon L, MD 12/15/2025 1:08 PM
Laceration Repair

Date/Time: 12/14/2025 12:20 PM

Performed by: Hua, Christine Tran, MD
Authorized by: Hua, Christine Tran, MD
Consent Obtained: Verbal
Consent given by: Patient
Risks and benefits: Risks, benefits and alternatives were discussed

Indication: Laceration

Consent: Prior to the procedure, the risks, benefits and alternatives were explained to the patient. The patient gave verbal consent to the procedure.

Timeout: Prior to the procedure, a time-out was called and correct patient and procedure were confirmed.

Site: left occipital scalp

Procedure: Prior to the repair, the patient's wound was cleansed. A total of 50 ml of normal saline was used to thoroughly irrigate the wound under pressure. The wound was inspected and no obvious foreign objects were found. The patient's wound was repaired with 6 staples. Total wound length was 3 cm. The patient tolerated the procedure well and there were no immediate complications.

Supervised Procedure: I am a resident/fellow and an attending was present to supervise the procedure
patient tolerated the procedure well with no immediate complications

Authorizing Provider	Result Type	Result Status
Christine Hua MD	IN CLINIC/BEDSIDE ORDERABLES	Final Result

X-Ray Chest Single View (X-RAY CHEST SINGLE VIEW) - Final result (12/14/2025 12:13 PM PST)

Anatomical Region	Laterality	Modality		
Chest		Digital Radiography		
Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
			12/14/2025 12:45 PM PST	

Impressions

12/14/2025 3:21 PM PST

Impressions
FINDINGS/

Frontal view chest. Clear lungs. No pleural effusion or pneumothorax. Normal cardiomediastinal silhouette.

No acute osseous abnormality.

Narrative

12/14/2025 3:21 PM PST
EXAM: X-RAY CHEST SINGLE VIEW

DATE: 12/14/2025 12:09 PM

HISTORY: SYNCOPE.

COMPARISON: None available at time of dictation.

Procedure Note

Dr. Anderanik Tomasian, MD - 12/14/2025
Formatting of this note might be different from the original.
EXAM: X-RAY CHEST SINGLE VIEW

DATE: 12/14/2025 12:09 PM

HISTORY: SYNCOPE.

COMPARISON: None available at time of dictation.

IMPRESSION:
FINDINGS/

Frontal view chest. Clear lungs. No pleural effusion or pneumothorax. Normal cardiomediastinal silhouette.

No acute osseous abnormality.

Authorizing Provider	Result Type	Result Status
Dr. Shannon Toohey MD	IMG XR PROCEDURES	Final Result

DIFFERENTIAL - UCI - Final result (12/14/2025 11:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Diff Type	Differential performed by automated analysis.			12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Neutrophils %	75.0	39.0 - 88.0 %		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Neutrophils Absolute	7.0	2.0 - 8.1 thous/ mcL		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Lymphocytes %	18.0	14.0 - 52.0 %		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Lymphocytes Absolute	1.7	0.9 - 3.3 thous/ mcL		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Monocytes %	5.6	1.0 - 11.0 %		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Monocytes Absolute	0.5	0.0 - 0.8 thous/ mcL		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Eosinophils %	0.9	0.0 - 7.0 %		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Eosinophils Absolute	0.1	0.0 - 0.5 thous/ mcL		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Basophils %	0.5	0.0 - 2.0 %		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Basophils Absolute	0.1	0.0 - 0.2 thous/ mcL		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Specimen (Source)	Anatomical Location / Laterality		Collection Method / Volume		Collection Time	Received Time
Blood	Peripheral blood / Unknown		Venipuncture / Unknown		12/14/2025 11:45 AM PST	12/14/2025 11:54 AM PST
Narrative						
Authorizing Provider	Result Type		Result Status			
Dr. Shannon Toohey MD	LAB BLOOD ORDERABLES		Final Result			
Performing Organization	Address		City/State/ZIP Code		Phone Number	
UCI MEDICAL CENTER	101 The City Drive		Orange, CA 92868			
HEMOGRAM, BLOOD - UCI - Final result (12/14/2025 11:45 AM PST)						
Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
WBC	9.3	4.0 - 10.5 thous/ mcL		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
RBC	5.09	4.38 - 5.62 mill/ mcl		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
HGB	15.3	13.5 - 16.9 g/ dL		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
HCT	45.7	39.5 - 50.0 %		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
MCV	89.7	81.5 - 97.0 fl		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
MCH	30.1	27.0 - 33.5 pg		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
MCHC	33.5	32.0 - 35.5 g/ dL		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
RDW-CV	13.0	11.6 - 14.4 %		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
PLT Count	227	150 - 400 thous/mcl		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
MPV	8.2	7.2 - 11.7 fl		12/14/2025 11:59 AM PST	UCI MEDICAL CENTER	
Specimen (Source)	Anatomical Location / Laterality		Collection Method / Volume	Collection Time		Received Time
Blood	Peripheral blood / Unknown		Venipuncture / Unknown	12/14/2025 11:45 AM PST	12/14/2025 11:54 AM PST	
Narrative						

Authorizing Provider	Result Type	Result Status				
Dr. Shannon Toohey MD	LAB BLOOD ORDERABLES	Final Result				
Performing Organization	Address	City/State/ZIP Code	Phone Number			
UCI MEDICAL CENTER	101 The City Drive	Orange, CA 92868				

TROPONIN I, HIGH SENSITIVITY (ED) - UCI - Final result (12/14/2025 11:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Troponin I, High Sensitivity	<3	0 - 20 ng/L		12/14/2025 12:25 PM PST	UCI MEDICAL CENTER	
Specimen (Source)	Anatomical Location / Laterality		Collection Method / Volume	Collection Time		Received Time
Blood	Peripheral blood / Unknown		Venipuncture / Unknown	12/14/2025 11:45 AM PST		12/14/2025 11:54 AM PST
Narrative						

UCI MEDICAL CENTER - 12/14/2025 12:25 PM PST
 These values were determined with a high sensitivity troponin I assay. The 99th percentile upper reference limits established by the manufacturer are:

Females: 15 ng/L
 Males: 20 ng/L
 Overall: 18 ng/L

Authorizing Provider	Result Type	Result Status				
Dr. Shannon Toohey MD	LAB BLOOD ORDERABLES	Final Result				
Performing Organization	Address	City/State/ZIP Code	Phone Number			
UCI MEDICAL CENTER	101 The City Drive	Orange, CA 92868				

(ABNORMAL) COMPREHENSIVE METABOLIC PANEL, BLOOD - UCI - Final result (12/14/2025 11:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Sodium	135 (L)	136 - 145 mmol/L		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Potassium	4.4	3.5 - 5.1 mmol/L		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Chloride	100	98 - 107 mmol/L		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Carbon Dioxide	28	21 - 31 mmol/L		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Comment: Samples with elevated Lactate Dehydrogenase (LDH) may cause falsely increased bicarbonate results. If elevated LDH levels are suspected, interpret bicarbonate results in the context of the patient's LDH values and overall clinical picture.						
Electrolyte Balance	7	2 - 12 mmol/L		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Glucose	112	70 - 115 mg/dL		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Comment: Normal Fasting Glucose: <100 mg/dL Impaired Fasting Glucose: 100-125 mg/dL Provisional DX of diabetes (must be confirmed) >125 mg/dL						
BUN	19	7 - 25 mg/dL		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Creatinine	1.0	0.7 - 1.3 mg/dL		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
eGFR	96	>=60 mL/min/1.73 sq mtr		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Comment: The reported eGFR is based on the CKD-EPI 2021 equation that does not use a race coefficient. The calculated GFR is an estimate and is NOT an accurate reflection of GFR in patients on dialysis. The accuracy of estimated GFR is also affected by muscle mass, and medications that affect the renal tubular secretion of creatinine. If estimated GFR will directly affect clinical decision-making, consider using cystatin C to estimate GFR, and/or consult with nephrology.						
Calcium	8.7	8.6 - 10.3 mg/dL		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Protein, Total	6.1	6.0 - 8.3 g/dL		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Albumin	4.3	4.2 - 5.5 g/dL		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Alkaline Phosphatase	49	34 - 104 U/L		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
AST	14	13 - 39 U/L		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
ALT	20	7 - 52 U/L		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Bilirubin, Total	0.4	<=1.4 mg/dL		12/14/2025 12:19 PM PST	UCI MEDICAL CENTER	
Specimen (Source)	Anatomical Location / Laterality		Collection Method / Volume		Collection Time	Received Time
Blood	Peripheral blood / Unknown		Venipuncture / Unknown		12/14/2025 11:45 AM PST	12/14/2025 11:54 AM PST

Narrative

Authorizing Provider	Result Type	Result Status	
Dr. Shannon Toohey MD	LAB BLOOD ORDERABLES	Final Result	
Performing Organization	Address	City/State/ZIP Code	Phone Number
UCI MEDICAL CENTER	101 The City Drive	Orange, CA 92868	

(ABNORMAL) PT/INR/PTT - UCI - Final result (12/14/2025 11:45 AM PST)

Component	Value	Ref Range	Test Method	Analysis Time	Performed At	Pathologist Signature
Prothrombin Time	11.9	10.2 - 12.9 sec		12/14/2025 12:22 PM PST	UCI MEDICAL CENTER	
INR	1.03	0.88 - 1.12		12/14/2025 12:22 PM PST	UCI MEDICAL CENTER	

Comment:
Therapy guidelines using the INR for patients stabilized on oral anticoagulant therapy:
 Moderate Intensity Regimen
 2.0 - 3.0 INR
 High Intensity Regimen
 2.5 - 3.5 INR
The physician must determine the best INR range depending on the reason for anticoagulant treatment and how each individual responds to treatment.
Reference: Chest 133:160S-198S, 2008

APTT	25.0 (L)	25.1 - 36.5 sec	12/14/2025 12:22 PM PST	UCI MEDICAL CENTER
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Comment:
Therapeutic Range for DVT or PE:
- Unfractionated Heparin: 1.5-2.5x of the mean aPTT reference range, 6 hours after bolus
- Argatroban: 1.5-3x of the mean aPTT reference range
- Please refer to latest pharmacy dosing protocol for current numeric ranges.

Specimen (Source)	Anatomical Location / Laterality	Collection Method / Volume	Collection Time	Received Time
Blood	Peripheral blood / Unknown	Venipuncture / Unknown	12/14/2025 11:45 AM PST	12/14/2025 11:54 AM PST

Narrative

Authorizing Provider	Result Type	Result Status	
Dr. Shannon Toohey MD	LAB BLOOD ORDERABLES	Final Result	
Performing Organization	Address	City/State/ZIP Code	Phone Number
UCI MEDICAL CENTER	101 The City Drive	Orange, CA 92868	

Visit Diagnoses - documented in this encounter

Diagnosis
Syncope and collapse - Primary
Laceration of occipital scalp, initial encounter
Closed head injury, initial encounter

Administered Medications - documented in this encounter

Inactive Administered Medications - up to 3 most recent administrations

Medication Order	MAR Action	Action Date	Dose	Rate	Site
acetaminophen (Ofirmev) IV injection 1,000 mg 1,000 mg, IntraVENOUS, ONCE, 1 dose, On Sun 12/14/25 at 1145, Administer over 15 Minutes, Do not refrigerate. Expires 6 hours after spiked.	New Bag	12/14/2025 11:55 AM PST	1,000 mg	400 mL/hr	
sodium chloride 0.9 % 500 mL IV bolus IntraVENOUS, ONCE, 1 dose, On Sun 12/14/25 at 1145, Administer over 30 Minutes, at 999 mL/hr	New Bag	12/14/2025 11:53 AM PST		999 mL/hr	

Insurance - documented as of this encounter

Guarantor: Dimarcello, Anthony Michael

Account Type	Relation to Patient	Date of Birth	Phone	Billing Address
Personal/Family	Self	05/04/1983		412 Olive Ave Huntington Beach, CA 92648-5142

COUNTY ADULT CUSTODY

Member	Subscriber	Plan / Payer (Effective 12/14/2025-Present)
Name: Dimarcello, Anthony Michael Member ID: 3390630 Relation to Subscriber: Self	Name: Dimarcello, Anthony Michael Subscriber ID: 3390630	Payer ID: Not on file Group ID: Not on file Type: CMS Phone: 800-206-6591 Address: PO BOX 13019 ATTN CUSTODY SERVICES LONG BEACH, CA 90853

Care Teams - documented as of this encounter

Team Member	Relationship	Specialty	Start Date	End Date
Per Patient No PCP	PCP - General	Internal Medicine	12/14/25	

Patient Contacts

Contact Name	Contact Address	Communication	Relationship to Patient
Ann Verma	412 Olive Ave HUNTINGTON BEACH, CA 92648	949-350-2312 (Mobile) amd949609@gmail.com	Other, Emergency Contact

Document Information

Primary Care Provider		Other Service Providers	Document Coverage Dates
Per Patient No PCP (Dec. 14, 2025 - Present)			Dec. 14, 2025
Internal Medicine			
UCI Health			
101 The City Drive South			
Orange, CA 92697			
Custodian Organization			
UCI Health			
101 The City Drive South			
Orange, CA 92697			
Encounter Providers		Encounter Date	
Dr. Shannon Toohey, MD (Attending)	Dr. Kristin Lewis, MD (Attending)	Dec. 14, 2025	
NPI: 1033418801	NPI: 1225590201		
714-456-7002 (Work)	714-456-7002 (Work)		
714-456-6085 (Fax)	714-456-6085 (Fax)		
101 THE CITY DR	101 THE CITY DR		
ORANGE, CA 92868	ORANGE, CA 92868		
Emergency Medicine	Emergency Medicine		
Legal Authenticator			
Background S			